

Accounting Terms & Concepts

Financial Management System (FMS — Transaction Core) • Quick Reference Handbook

Purpose of this Handbook: This guide explains the core financial and accounting terminology used in our Hospital Financial Management System (FMS). It connects technical accounting concepts directly to everyday hospital operations (pharmacy, patient billing, doctor fees, and supplier payments) with clear real-world examples.

The Fundamental Accounting Equation: **ASSETS** = **LIABILITIES** + **EQUITY**

1 The 5 Major Financial Account Categories

In our Chart of Accounts (COA), every peso in the hospital belongs to one of these five primary groups:

CATEGORY	COA CODE	NORMAL BALANCE	DEFINITION	HOSPITAL REAL-WORLD EXAMPLE
Asset	1000-1999	DEBIT	Valuable resources that the hospital owns or controls.	Cash in bank accounts, pharmacy medicine inventory, ultrasound/CT machines, patient receivables.
Liability	2000-2999	CREDIT	Debts or financial obligations the hospital owes to others .	Unpaid bills to medicine suppliers (Unilab), taxes withheld for BIR, accrued doctor professional fees.
Equity	3000-3999	CREDIT	The net worth of the hospital (Assets - Liabilities).	Owner's starting capital, retained earnings from previous years of hospital operations.
Revenue (Income)	4000-4999	CREDIT	Money earned by the hospital from providing medical services.	Inpatient room fees, outpatient consultation fees, laboratory test fees, pharmacy drug sales.
Expense	5000-5999	DEBIT	Money spent on day-to-day operations to keep the hospital running.	Nurses and doctors' payroll salaries, electricity and water utilities, disposable medical gloves, ambulance diesel.

2 Double-Entry Bookkeeping Mechanics

Debit (Dr) vs. Credit (Cr)

Core Engine Rule

Double-entry accounting requires that every financial transaction has two sides. A **Debit** increases Assets and Expenses (while decreasing Liabilities, Equity, and Revenue). A **Credit** increases Liabilities, Equity, and Revenue (while decreasing Assets and Expenses).

Real-World Example: When a patient pays ₱2,000 cash for laboratory blood tests:

- *Debit:* Cash in Bank (Asset increases) = ₱2,000
- *Credit:* Laboratory Service Revenue (Revenue increases) = ₱2,000

Debit-Credit Invariance Rule

Mathematically Enforced

In our system, **Total Debits must exactly equal Total Credits** ($\Sigma \text{Debit} = \Sigma \text{Credit}$) on every single transaction. If there is a discrepancy of even 1 centavo, the transaction cannot be saved or posted to prevent financial corruption.

General Ledger (GL) & Journal Entries

Central Record

A **Journal Entry** is an individual record of a transaction. The **General Ledger (GL)** is the master collection of all journal entries that forms the official financial book of the hospital.

Reversal Entry (Immutability Policy)

Audit Protection

Once a financial record is posted, it **cannot be deleted or edited**. If an error was made, the system creates a compensating **Reversal Entry** (swapping Debits and Credits) to cancel it out cleanly with an audit record for government inspectors.

3 Patient Billing & Accounts Receivable (AR) Terms

Accounts Receivable (AR)

Money Inbound

Money owed to the hospital by discharged patients, private insurance HMOs, or government agencies for medical services already delivered.

Real-World Example: A patient was discharged on Tuesday. Their total bill was ₱30,000, and their HMO promised to pay within 30 days. That ₱30,000 sits in **Accounts Receivable — HMO Claims** until the check clears.

AR Aging Buckets

Credit Risk Analysis

A tracking table that groups unpaid patient and insurance bills by how long they have been outstanding: Current (0–30 days), 31–60 days, 61–90 days, and Over 90 days (Past Due / High Risk).

Statutory Discounts (RA 9994 Senior Citizen & RA 10754 PWD)

Philippine Law Compliance

Philippine statutory mandate: When an eligible Senior Citizen or PWD patient is billed, the system first exempts the 12% Value Added Tax (VAT Relief = Gross / 1.12), then deducts an additional **20% discount** on professional fees and medicines.

Real-World Example: A ₱1,120 medicine bill for a Senior Citizen becomes ₱1,000 Net of VAT, minus 20% discount (₱200) = Patient pays only **₱800**.

Third-Party Payers (PhilHealth, HMO & Malasakit Assistance)

Payer Integration

Organizations that pay hospital bills on behalf of the patient:

- **PhilHealth ACR (All-Case-Rate):** Standard fixed payment package from the Philippine government (e.g. ₱15,000 for Appendectomy).
- **HMO (Health Maintenance Organization):** Private insurance providers (Maxicare, Intellicare) issuing a **Letter of Authorization (LOA)** with a set coverage ceiling.
- **Malasakit Center (RA 11463):** Issues **Guarantee Letters (GL)** from DOH-MAIP, PCSO, or DSWD to sponsor indigent patients.

4 Supplier & Accounts Payable (AP) Terms

Accounts Payable (AP)

Money Outbound

Money the hospital owes to external suppliers for goods or services delivered on credit.

Real-World Example: The hospital orders ₱80,000 worth of dialysis supplies from Baxter. Baxter delivers the supplies, and the hospital has 30 days to pay.

3-Way Matching (PO ↔ GRN ↔ Vendor Bill)

Anti-Fraud Safeguard

The mandatory system verification comparing:

1. **Purchase Order (PO):** What Procurement originally ordered.
2. **Goods Receipt Note (GRN):** What the warehouse loading dock physically received and verified.
3. **Vendor Invoice:** What the supplier is charging the hospital.

If all 3 quantities and prices match, the bill is approved. If there is a price or quantity variance, it is held.

5 Cash Management & Disbursements

Disbursement Voucher (Payment Voucher)

Payment Authorization

The internal document prepared by accounting and signed by the CFO authorizing the release of hospital funds to pay a supplier, doctor, or staff expense.

Petty Cash Fund (Imprest System)

Fast Cash Reserves

A small, fixed fund of cash (e.g. ₱10,000) kept in a secure vault at the hospital cashier to pay for minor immediate expenses (like emergency pharmacy runs, mail stamps, or drinking water refills). When depleted, receipts are submitted to replenish it back to ₱10,000.

Electronic Fund Transfer (EFT) & Check Register

Payout Channels

Methods of payment: **EFT** is a paperless bank-to-bank transfer directly into the supplier's account. A **Check Register** logs printed bank checks with check numbers, issue dates, payee names, and clearing statuses.

Bank Reconciliation

Treasury Audit

The monthly process of comparing the hospital's internal ledger with the official bank statement feed to verify deposits, cleared checks, and bank charges to the exact centavo.

6 Budgeting & Financial Control Terms

Budget Allocation vs. Encumbrance

Spending Guardrails

- **Budget Allocation:** The maximum spending limit assigned to a department for the year (e.g. ₱3,000,000 for Surgery).
- **Encumbrance (Commitment):** "Freezing" or reserving money from that budget as soon as a Purchase Order is approved, preventing the money from being spent twice while waiting for delivery.
- **Liquidation:** Converting the encumbrance into an actual expense once the bill is paid.

7 Taxation & Philippine BIR Regulatory Terms

Expanded Withholding Tax (EWT) & BIR Form 2307

Tax Compliance

The hospital is legally mandated to deduct withholding tax from supplier payments (1%–2%) and doctors' professional fees (10%–15%) and remit it to the Bureau of Internal Revenue (BIR). The hospital issues **BIR Form 2307** to vendors as official proof of tax withheld.

Computerized Accounting System (CAS) Audit Trail

Anti-Tampering Trail

BIR regulations requiring all software modifications, login shifts, and ledger entries to have an unalterable, sequential log (`cas_audit_trails`) recording the user ID, workstation IP, exact timestamp, and old vs. new values.

8 Key Financial Statements & Executive Reports

REPORT NAME	WHAT QUESTION IT ANSWERS	HOSPITAL EXECUTIVE DECISION
Trial Balance	Are all debit and credit accounts in mathematical balance?	Accountants use this before closing the monthly books to ensure zero discrepancies.
Balance Sheet (Statement of Financial Position)	What is the hospital's overall financial health and net worth on this specific day?	Bankers and Board of Directors evaluate the hospital's liquidity, solvency, and debt obligations.
Profit & Loss (P&L) (Income Statement)	Did the hospital make a profit or run at a financial loss over the last month/year?	CFO and Hospital Director decide if prices need adjusting or departmental costs need cutting.
Cash Flow Statement	Where did the hospital's cash come from and where was it spent?	Ensures the hospital has enough liquid cash in the bank to pay salaries and supplier bills on time.

Hospital Financial Management System (HIMS • FMS) — Prepared for College Capstone / Systems Defense • Generated October 2026